A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

29 April 2026

Dr David Kosenko
Ti-Tree Family Doctors

Dear Dr Kosenko,

**Re: Mrs Leocadia Davey**
**DOB: 20/07/1955 Phone: 0408 148 481**
**Address: 136 Koornalla Crescent, MOUNT ELIZA, VIC, 3930**
**Email: leocadiajohn@hotmail.com**
**Medicare: 3010909933 / 1**

**Summary**

- **Improvement in symptoms of dysphagia and bolus obstruction following adjustment of medical therapy.**
- **Recent episodes of severe retrosternal chest pain and exertional dyspnoea.**
- **Action required: Please consider arranging a cardiac evaluation to exclude ischaemic heart disease given her age and exertional symptoms.**

I had the pleasure of conducting a clinical review with Leocadia Davey. She reports a general improvement in her symptoms since her last review in February. Most notably, the sensation of food getting stuck in the distal oesophagus has largely resolved, and she is no longer experiencing the frequent dysphagia that prompted her previous visit.

Leocadia is currently taking omeprazole in the morning and nizatidine at night. She finds the addition of the nizatidine particularly beneficial, though she has noted a significant reduction in saliva production, which is a known pharmacological effect of the medication. She also continues to use a topical steroid as part of her management.

The focus of our consultation today was two recent episodes of retrosternal chest pain. The most severe episode occurred last week while she was walking her dog and was significant enough to cause a sensation of breathlessness. While she has a history of negative cardiac investigations from four years ago, she has noticed increasing dyspnoea when walking uphill over the last six months. She also noted that these episodes can occur during periods of increased psychological stress, though they are not exclusively tied to her emotional state.

**Impression and Plan**

While it is possible that these episodes represent oesophageal spasm related to her known motility disorder, the exertional nature of the pain and the associated breathlessness in a 70-year-old patient necessitate further investigation. I have advised Leocadia that we must exclude a cardiac cause before attributing these symptoms solely to her oesophagus or to stress. I would be grateful if you could facilitate a cardiac workup - if you also agreed.

All appointments and enquires
**Suite 201, 100 Victoria Parade**
**East Melbourne VIC 3002**
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
**Epworth Freemasons Day Proc. Centre**
320 Victoria Pde, East Melbourne, 3002
**Hobson Healthcare Sydenham**
566 Melton Hwy, Sydenham, 3037
**St Vincent's Private Hospital**
59 Victoria Pde, Fitzroy, 3065

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In the interim, we will maintain her current gastrointestinal medication regimen. We discussed the potential use of sublingual nitrates as a smooth muscle relaxant for acute spasm; however, I am hesitant to initiate this currently due to the risk of hypotension and headaches, especially while her cardiac status is being clarified. Should the cardiac investigations prove negative and the spasms continue to be debilitating, we may consider further physiological testing such as repeat manometry. I have planned to review Leocadia in four months to reassess her progress and discuss the results of her cardiac investigations.

Yours sincerely,

**A/Prof Chamara Basnayake**
Gastroenterologist

CC: A/Prof Bronte Holt


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